Nursing Essentials

Priority: Interpret bilirubin by age in hours, gestational age, and neurotoxicity risk—not by one universal cutoff.

FOCUS 1

Recognize

- **First 24 hours:** jaundice warrants prompt evaluation for an underlying cause.
- **Assess:** feeding, weight trend, hydration, stool / urine output, bruising, and risk for hemolysis.
- **Urgent neurologic signs:** poor feeding, abnormal tone, marked lethargy, or a high-pitched cry require escalation.

FOCUS 2

Investigate

- **Mechanism:** faster red-cell breakdown increases bilirubin production and can cause anemia.
- **Possible causes:** blood-group incompatibility, G6PD deficiency, and other disorders; a DAT is only part of the evaluation.
- **Rh immune globulin:** helps prevent Rh sensitization in eligible unsensitized clients; it does not prevent ABO incompatibility.

FOCUS 3

Treat

- **Use the prescribed setup:** correct device, exposure, positioning, and eye protection when required.
- **Monitor:** temperature, feeding, hydration, weight, output, and ordered bilirubin measurements.
- **Support feeding:** assess milk transfer and intake; follow the plan for supplementation or fluids when indicated.

Teach and follow through

Make it stick: The AAP hyperbilirubinemia guideline for infants at least 35 weeks does not replace a specialized pathway for younger preterm infants.

Focus	What to know	Nursing application
Follow-up timing	based on bilirubin level, treatment threshold, age, risk, and the discharge plan.	Schedule reassessment
Do not rely on appearance	skin assessment alone cannot determine a treatment threshold.	Obtain the indicated level
No sunlight treatment	use recommended medical treatment and scheduled reassessment.	Use medical treatment

RED FLAGS · ESCALATE PROMPTLY

Jaundice in the first 24 hours, poor feeding with marked lethargy, abnormal tone, or a high-pitched cry warrants prompt evaluation.